Draft: pending clinical review

Checking your blood sugar at home

How to check, how to write it down, and when to get help: with a record to bring to every visit

The 5 things to remember

- **Wash and dry your hands** before every check.
- Write down **every** reading with the time, and whether it was before or after a meal.
- **Below 70**: take 15 g of fast sugar, then check again after 15 minutes.
- When sick: check **every 4 hours**, keep drinking, and **never stop insulin**.
- Bring this record, your meter and your medicines to **every visit**.

When should I check?

My treatment	Usual checks: your doctor may change this
Insulin with meals (several injections a day)	Before meals and at bedtime; sometimes 2 hours after meals.
Long-acting insulin once a day, or gliclazide or glimepiride	Before breakfast every day, and whenever you feel low or unwell.
Other tablets or weekly injections only	As your doctor advises, often a few times a week.
Any treatment	When you feel shaky, sweaty or confused; more often when sick or fasting (see the Ramadan leaflet). Before driving if you use insulin, gliclazide or glimepiride.

How do I check?

- Wash your hands with soap and water and dry them: sugar on your fingers gives a false high.
- Use the side of a fingertip, and change fingers each time.
- Use strips that are in date. Use a new lancet each time, never share it, and put used lancets in a hard plastic bottle.
- Using a sensor (CGM)? Bring the app report, and keep your meter to check when a reading does not match how you feel.

What are my targets?

When	Usual target for many adults	My target (set by my doctor)
Before meals	80–130 mg/dL (4.4–7.2 mmol/L)	
1–2 hours after starting a meal	Below 180 mg/dL (10.0 mmol/L)	

Older adults and people prone to lows often have higher targets.

Low sugar: below 70 mg/dL

- Signs: shaking, sweating, hunger, fast heartbeat, confusion. Some people feel nothing: check if unsure.
- Take **15 g of fast sugar**: 3 teaspoons of sugar in water, half a cup (120 ml) of regular juice, or glucose tablets. Always carry some with you.
- Check again after **15 minutes**. If still below 70, repeat. When it is above 70, eat a snack or your meal.
- Write every low on page 4, and tell your doctor.

Sick days

- Check your sugar **every 4 hours**, day and night.
- Drink a glass of water or a sugar-free drink every hour.
- **Never stop insulin**: you may need more.
- Some tablets may need a pause while you cannot eat or drink, such as metformin, dapagliflozin or empagliflozin, and some blood pressure or water tablets. Only do this if your doctor has given you a sick-day plan.
- Type 1 diabetes, or sugar above 300: check ketones if you have strips.

NOW **Call 997 or go to the emergency department**

- Low sugar that does not improve after 2 treatments, or the person is confused, very drowsy or fitting. Do not give food or drink to someone who cannot swallow.
- Vomiting, and you cannot keep fluids down.
- Sugar above 300 with tummy pain, vomiting, fast deep breathing or drowsiness, or medium or high ketones.
- On dapagliflozin or empagliflozin with vomiting, tummy pain or fast breathing: **even if your sugar is normal**.

TODAY **Contact your clinic today (or call 937)**

- Sugar stays above 250, or before-meal readings stay above 240 for more than a day.
- Fever or illness for 2 days, or vomiting or diarrhoea for more than 6 hours.
- A low below 54, a low that needed someone's help, or 2 or more lows in a week.

NEXT VISIT **Bring to your next visit**

- Most readings above your target.
- Questions about your medicines, food, activity, or whether a sensor (CGM) could help you.

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This leaflet gives general information. It does not replace advice from your own doctor, who knows your health. Bring it to your next visit and ask about anything that is unclear. Written with AI assistance, checked by the medical editor, and still under review with colleagues.

Last updated: October 2026 · **Next review:** October 2027, or sooner if guidance changes

Sources:

1. American Diabetes Association. Standards of Care in Diabetes: 2026, Sections 6 (glycemic goals, hypoglycemia, hyperglycemic crises, intercurrent illness) and 7 (diabetes technology). Diabetes Care 2026;49(Suppl 1); Summary of Revisions doi:10.2337/dc26-SREV
2. Saudi Ministry of Health. Diabetes and being sick: health awareness (moh.gov.sa)
3. Joint British Diabetes Societies. Sick day rules and SGLT2 inhibitor patient guidance (2025)
4. IDF-DAR Practical Guidelines: Diabetes and Ramadan, 2021; 2026 risk calculator update

My blood sugar record

BRING TO EVERY VISIT

Name: _____ File number: _____

Type of diabetes: _____ My meter: _____

My medicines and insulin doses: _____

My targets (written by my doctor): _____

Write readings in mg/dL. **Circle** any reading below 70. Write insulin doses or changes in the notes.

Date	Before breakfast	After breakfast	Before lunch	After lunch	Before dinner	After dinner	Bedtime	Notes / insulin
15/10	*118*				*142*		*126*	*lunch out*

For the clinic

Readings below 70 / below 54: ___ Average before breakfast: _____ Reviewed by / date: _____

Low sugar and sick days

BRING TO EVERY VISIT

Low sugar record (below 70)

Date and time	Reading	What happened / how I felt	What I took	Reading after 15 min	Needed help? (yes / no)

Sick-day record

Date and time	Sugar	Ketones	Drinks in the last 4 hours	Medicines and insulin taken	Notes

Questions for my next visit

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